2.15 IV Therapy and Central Venous Lines

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "The area around the cannula is sore." **Objective** • Cannula site red and swollen • Skin around the site cool and pale • Drip running slower	**ACTUAL** **Impaired tissue integrity** related to infiltration or phlebitis as evidenced by swelling, redness and pain at site.	Within 48 hours the site has healed, with no redness or swelling.	• Stop the infusion and take the cannula out. • Raise the limb and apply a compress as per policy. • If a new cannula is needed, put it in the other limb. • Grade the infiltration or phlebitis and document it.	Site healing: **goal met**.
Subjective • "What should I do if the line pulls?" **Objective** • PICC line just inserted • Cannot explain how to look after the line	**ACTUAL** **Deficient knowledge** related to care of vascular access device as evidenced by questions about line care.	Before discharge she can name the warning signs and explain how to protect the line.	• Teach her to keep the dressing dry and intact, and to cover it in the shower. • Teach the signs of infection (redness, discharge, fever) and of a blocked line. • Tell her not to pull or bend the line, and not to let anyone else use it. • Give written instructions and a number to call.	Explains line care correctly: **goal met**.
Subjective • *No specific complaint* **Objective** • Central venous catheter • On IV therapy long term	**RISK** **Risk for infection** as evidenced by risk factors: invasive vascular device.	No CLABSI.	• Hand hygiene and aseptic technique every time the line is accessed. • Scrub the hub before each access. • Change the dressing as per policy, or sooner if it is soiled. • Look at the site every day and ask whether the line is still needed. • Watch the temperature.	No signs of infection: **goal met**.